

Hair Loss - Female (Adult)

Algorithm 1100 | Adult (18 years and older) | Gender at birth: Female

Definition

- Hair loss assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Adult patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, are they pregnant/recently postpartum, and when/how quickly did hair loss begin? This protocol starts at age 12; younger children require an approved pediatric route.
2. [OPEN_TEXT] Is it patchy, all over, or a receding pattern?
3. [OPEN_TEXT] Is there scalp pain, redness, swelling, pus, blistering, chemical/thermal exposure, new medicine, fever, weight loss, restrictive eating, endocrine symptoms, pregnancy/postpartum change, hair pulling/traction, distress/self-harm concern, or a safeguarding concern?

Triage and redirect questions

1. Disposition level 70

Is an inflamed, painful, scaling, crusting, or infected-looking scalp the main concern rather than hair loss itself?

The localized-rash family is not generated. Arrange prompt in-person assessment through the Qatar route approved by governance (GOVERNANCE_REQUIRED).

1. Disposition level 100

Is hair/scalp change part of a current emergency: chemical or electrical exposure, major burn, facial/airway swelling, breathing difficulty, collapse, severe infection with confusion, poisoning/overdose, severe dehydration/malnutrition, or immediate self-harm/safeguarding danger?

Hair loss alone is generally not an emergency, but the reported presentation may be secondary to a time-critical exposure, allergic reaction, infection, poisoning, malnutrition, mental-health crisis, or safeguarding threat.

1. Disposition level 50

Is the caller worried about the hair loss and would like it assessed?

NHS.UK guidance: see a GP if you're worried about your hair loss, to identify the cause before considering commercial hair clinic options.

Advice

Qatar emergency response for the underlying danger

- Call Qatar emergency services on 999 and do not drive. Follow the relevant exposure, burn, anaphylaxis, poisoning, severe-illness, suicide-safety, or safeguarding instructions from 999. Do not apply chemicals, attempt decontamination beyond emergency instructions, or give unprescribed medicine. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: airway or breathing change; collapse, confusion, or severe illness; chemical/burn exposure; self-harm or safeguarding danger.

Governed in-person hair-loss assessment

- Arrange an in-person assessment through the Qatar route and timing approved by governance to evaluate pattern, scalp disease, medicines, nutrition, endocrine illness, pregnancy/postpartum factors, traction/pulling, and psychosocial impact. Do not start supplements, topical medicines, or hair-loss drugs without age, pregnancy, diagnosis, interaction, and laboratory review. For an adolescent, provide private age-appropriate assessment with approved caregiver and safeguarding practice.
- Call Back If: rapid or patchy progression; scalp pain, inflammation, pus, or scarring; weight loss, restrictive eating, systemic illness, or severe distress.

First aid

- Call Qatar emergency services on 999 and do not drive. Follow the relevant exposure, burn, anaphylaxis, poisoning, severe-illness, suicide-safety, or safeguarding instructions from 999. Do not apply chemicals, attempt decontamination beyond emergency instructions, or give unprescribed medicine. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Hair loss", <https://www.nhs.uk/conditions/hair-loss/> (page last reviewed 24 January 2024)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from the cited NHS.UK pages under the Open Government Licence v3.0; it is not licensed STCC content.

Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Local extensions, destination mappings, safeguarding workflows, and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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